

Meeting Transcript — September 10, 2026

Participants: Isak Gutierrez, Thomas Abshier, Joelle Ater (joined ~9:03 AM) **Duration:** ~3.5 hours (7:18 AM – ~10:16 AM)

Part 1 — Soft Goods Reality Check (~7:18–7:30)

Thomas: It seems like if we're actually planning on being successful and having a company, we're not even close to having a company right now. The only value in having your mom sew is having one so we can do photos and demos and create the website.

Isak: Yeah, it is the next step, but it's not the ideal for actual production.

Thomas: I just couldn't believe it would take two weeks. I thought maybe a day or two. We were even thinking of having her be the one who was production, but it's not even close.

Isak: Two weeks is a liberal estimate. I wanted to give her room. The cutting and measuring is a huge portion — if we do bulk cuts assembly-line style, it could be a few days per set. But for this first one, it's kind of a prototype. She's happy to not do it long-term — she said "don't worry about offending me."

Thomas: Your mom is into Bible study, not into being a seamstress. We need to find somebody else. But having her create one that meets our spec is a real advantage — we send that to any contract manufacturer and say "make it look like this."

Isak: Think about Mom and Dad as the prototype people, and think about everybody else as Victory Circle and seamstresses everywhere.

Part 2 — DME Application Urgency (~7:31–7:33)

Thomas: Did you get the urgency of doing the DME application from yesterday's analysis? Between January and 18 months ago, they had it frozen. They just unfroze it 13 days ago.

Isak: I remember. So there's people flooding in to apply, which means they could shut it down again.

Thomas: The idea that we'd be able to sell it for a million dollars is actually dependent on government-created scarcity. That scarcity might go away. We can't count on it being a million dollar payday if we don't use it.

Part 3 — Rifton's Model & Reimbursement Reality (~7:51–7:55)

Isak: (*reading analysis*) Rifton sells through DME dealers heavily — that's their model. They publish sample letters of medical necessity for every product. Everything billed under E1399, miscellaneous DME. The engine underneath is pediatric EPSDT.

Price stretch: Rifton reimburses at \$3,000–\$5,000. Biodex does not reimburse at \$12,900. We're at \$12,000–\$16,000 trying to reimburse — that's one combination neither of them does.

Rifton uses the caregiver argument inside the medical necessity letter: "the device has the potential of saving future medical costs of caregivers injuring their backs during lifting and transfers."

Part 4 — DME Economics Deep Dive (~7:39–8:10)

Thomas: (*sharing screen*) Setup cost year one: CMS enrollment \$750, accrediting organization \$1,500–\$5,000, policies and procedures \$1,000–\$5,000, surety bond \$1,500, state license \$2,000. Hard floor year one: \$4,000–\$10,000. Annual renewal: \$2,000–\$7,000.

Thomas: So basically \$10,000 first year, \$7,000 per year after. Plus a location. How much for a 200 sq ft space in Bakersfield?

Isak: Closed-door business is explicitly permitted. Flex industrial at \$13–\$19/sqft. Monthly about \$750 for a 600 sqft space.

Thomas: So \$20,000 a year for rent. Then an employee — California minimum is \$16.90. With taxes, worker's comp, the actual cost is closer to \$20/hour. For 30 hours a week, that's about \$39,000–\$43,000 a year.

So \$60,000 a year just to have the DME available. Plus if we sell 15 a month at \$3,000 profit per unit through other DMEs, that's \$45,000/month. Minus \$16,000 payroll leaves \$30,000 actual monthly margin.

Part 5 — The Hybrid Plan (~8:24–8:30)

Thomas: Let's get DMEs all over the country selling our product — that's what Rifton's done. We're fitting into an established market that's not yet saturated.

We can establish relationships with every DME in the country. They're all selling our stuff. In the meantime, we're becoming their competition, but they're supporting us. We say "you can get it from a local DME" and then arrange the sale.

Cayenne could be coordinating that. Joelle could be doing it too. We have a sales staff of two or three.

The virtual assistant creates a warm market by getting responses, educating them. Then we call them up — "did you see the walkthrough? What did you think? Do you have patients who need this?"

Isak: I like that. It's a mix of the big enterprise and the bootstrap startup.

Part 6 — Joelle Joins, DME Strategy Confirmed (~9:03–9:20)

Isak: (*summarizing for Joelle*) The plan changed. We're doing hybrid — selling to every DME we can, routing incoming leads to whichever local DME serves that buyer. Virtual assistant for lead

gen, us doing follow-up calls.

Joelle: Would we be doing outreach to doctors first, or starting with the DMEs?

Isak: Probably getting info from DMEs ahead of time and getting some leads. A bit of both.

Joelle: Don't you need DMEs in every region?

Isak: Yeah, but that's the model. Rifton sells through Adaptive Mall, SpinLife, Rehabmart, Medical eShop. They don't own a DME — they publish funding materials and let local dealers handle it.

Thomas: Can we sell nationwide from a California DME?

Isak: *(reading)* Medicare yes — DME claims go to the MAC for the jurisdiction where the beneficiary lives, not where the supplier sits. But state licensure — no. Many states require out-of-state suppliers to register. And Medicaid — definitely no. 50 separate programs, 50 enrollments.

This is why Rifton doesn't own a DME. The hybrid plan already solves this — you need a network of locally licensed dealers. Owning a California DME would let you serve California well and everywhere else badly.

Part 7 — Logo, CRM, Notion (~9:33–9:46)

Joelle: I started the CRM in Airtable and a project hub in Notion. Also have a logo concept — simple, the O has a swirl suggesting motion. Did it in Canva.

Thomas: It's very professional. Serious. Not whimsical.

Isak: The circle says motion twice — from the word and from feeling it.

Joelle: Charlie approved via text. The Keep On Truckin' design could be used for stickers and accessories.

Thomas: So we probably should go with serious. It implies stability, gravity, long-term quality.

Part 8 — Soft Goods Outsourcing (~9:30–10:04)

Joelle: We didn't figure out pricing on soft goods. 15 units is not sustainable for Susan.

Isak: We need to talk to Leonard about that. And check with you about your backpack contact.

Joelle: He never got back to me. I can pursue different routes. I have a Facebook group with every business owner in the area — I can find local seamstresses fast if I have a picture and basic specs.

Isak: I'll visit Mom and Dad today, collect patterns, photos, materials list. They have paper patterns I can photograph. And I need to check if Charlie has CAD drawings for soft goods.

Thomas: I think your dad already drew up CAD drawings of all the soft goods.

Isak: They're not in a shared folder anywhere. I'll check with him.

Joelle: Even just a basic list — materials, alternate versions, sizes — so I can get quotes.

Thomas: Having a physical set that we say “duplicate this” will save heartache. We had issues before where manufacturers got the thickness, padding, and feel completely wrong.

Part 9 — Final DME Decision (~10:05–10:16)

Thomas: Let’s examine it once more. At what point after applying do you need the facility set up?

Isak: *(reading)* The facility has to be real BEFORE the application. CMS eliminated the three-month grace period January 1, 2026. The actual sequence: Month 0 — lease, build out, signage, landline. Month 0–1 — person hired and working. Month 1–2 — apply to accrediting organization. Month 3–6 — unannounced survey. Month 6 — file CMS 855S. Month 9–12 — PTAN issued, first billable claim. Roughly 9–12 months of paying rent and wages before a dollar comes back. \$45,000–\$60,000 spent before first claim.

Isak: Wholesale to DMEs does NOT cap future billing — it’s a trade price. Direct cash sales to families ARE what set the floor. So wholesale freely, be deliberate about direct-to-consumer pricing.

Thomas: I was thinking it was nationwide, but it’s not. That was simply wrong. Is it worth being a DME for the extra sales in California alone? Right now, we’re in no position to capitalize on that.

Isak: It really comes down to what Medi-Cal and other insurers actually pay. Otherwise we’re just swimming in circles. I’m gonna get a hold of these guys.

Thomas: Get a hold of it. The DME route through other dealers, letting them take their cut — I think when we finally get mature, we can get our own DME for California. But we have to hold off for now.